



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صفاء فتوح السيد سليمان
المحافظة : القاهرة
الحى / المدينة : عين شمس
رقم البطاقة : 27501010105863 رقم التليفون : 01125811104
العنوان : =
العمر (تاريخ الميلاد) : 01-01-1975 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: صفاء فتوح السيد سليمان National ID: 27501010105863	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
مفاتيح



Name : Safaa Fattouh Al Sayed
Branch : Portable
Ref.Physician Prof : CAP+C

Date : 16-02-2026 01:40
Medical No. : GRI-12243
Ins.Company :

**MULTI-DETECTOR C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS FOLLOWING ORAL AND
INTRAVENOUS CONTRAST MEDIA ADMINISTRATION
REVEALED:**

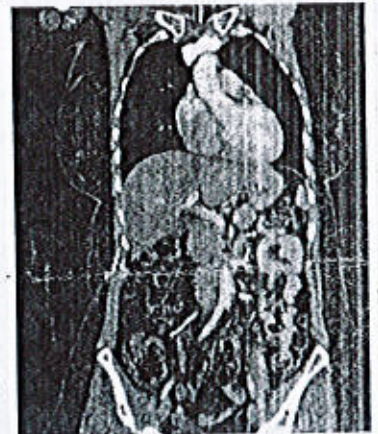
Clinical data: Breast cancer as compared with previous CT dated 7/10/2025 the current study reveled

Stationary course with no interval changes

- Normal lung parenchyma, showing no focal lesions.
- Normal vascular mediastinal structures.
- The trachea, main stem bronchi as well as their lobar and segmental divisions are normal.
- No evidence of mediastinal or hilar nodal enlargement .
- No pleural collections.
- No detectable lesions in the chest wall.
- The liver is of normal size, and tissue density. It shows no focal or enhancing lesions.
- The intra and extra-hepatic biliary radicles are normal.
- The pancreas and spleen are normal.
- Normal CT features of both kidneys.
- Right and left supra renal glands are normal.
- No CT evidence of enlarged abdominal or pelvic lymph nodes.
- No ascites.
- Scans through the pelvis showed no masses.
- No suspicious bony lesion

Opinion

Stationary course with no metastatic deposits



**Much Obliged;
Dr.Khaled Moghazy,MD
Prof.of Radiology**

Patient Name: Safaa Fattouh Al Sayed Soliman	Date : 07/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 509/2021	Pat. ID: 12243

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to 20.07.2024 the current study shows:

Chest:

- Rather stable course regarding the size of few right axillary level I lymph node being subcentimeteric in short axis.
- still seen left lower lung lobe atelectatic band
- Still noted right pulmonary nodules , the largest seen at right upper lobe posterior segment measuring about 7.5 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen and pelvis:

- The liver is average in size with fatty infiltration of its parenchyma. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Prominent extra renal pelvis is noted on the right side
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- The scanned skeleton shows no worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up study showing:

1. ***Rather stationary course regarding the previous study.***

DR. Prof. Ali Remah

Patient Name: Safaa Fallouh Al Sayed Soliman	Date : 01/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 509/2021	Pat. ID: 12243

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to 20.07.2024 the current study shows:

Chest:

- Rather stable course regarding the size of few right axillary level I lymph node being subcentimetric in short axis.
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- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Prominent extra renal pelvis is noted on the right side
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- The scanned skeleton shows no worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up study showing:

- Overall stationary course.

Dr/Reham Khedr, MD

CT Scan
[DOCT/12243_CT_SCAN]

Patient Name: Safaa Fattouh Al Sayed Soliman	Date : 15/01/2025
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 501/2021	Pat. ID: 12243

Combined bilateral breast mammography and complementary ultrasound

Indication:

Follow up after chemotherapy for right breast cancer

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast show subtle architectural distortion seen at UOQ about 20x15mm with underlying microcalcifications..
- No spiculated masses or suspicious microcalcifications on the left breast.
- Bilateral benign looking calcifications.
- Complementary ultrasound revealed right breast no appreciated any suspicious lesion seen as fibrotic band.
- Normal skin thickness, and nipple-areola complex of the left breast.
- No pathological right axillary or supraclavicular lymph nodes.
- Nonspecific left axillary LN.

Opinion:

- Right breast pathologically proved carcinoma on NACTH with complete radiological response (BIRADS 6)
- Benign findings left breast (BIRADS 2)

Signature Dr. Nehal Hamdy

15/01/2025 1:00 PM

Patient Name	: الأستاذة / صفاء فتوح السيد مكيومان	Request No.	: 13003675
Patient No.	: 27501010105863	Request Date	: 11/03/2024
Ref No.	: 995/24	Verification Date	: 25/03/2024
Sex	: Female	Financial Category	: مبداءة - مزورم دار السلام
Age	: 49 Y	Department	: Out Patient
Referring Doc		IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Specimen Data: Right breast mass.

Specimen Description: Tru-cut biopsy, collected on 10/3/2024.

Technique

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machines (Ventana)

and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and

hematoxylin as counter stain. The sections treated against:

ER clone (1E2), PR clone (SP-1), Her2 clone (4B5), & Ki67 clone (30-9).

Results

ER: Positive control showed appropriate reactivity):

ER: Weak nuclear reaction in 8% of the tumor cells (score 3/8).

PR: Weak nuclear reaction in 70% of the tumor cells (score 6/8).

Her2: Strong complete membrane reaction in >10% of the tumor cells (score 3+).

Ki67: 50% nuclear reaction of tumor cells.

Conclusion

Right breast mass; Tru-cut biopsy:

Invasive duct carcinoma, Grade II with focal DCIS.

PT

ER: Positive (score 3/8).

PR: Positive (score 6/8).

Her2: Positive (score 3+).

Ki67: Positive in 50% of tumor cells.

Stain system: "ER, PR"

Performed by

Dr. Manna Nabil

Page 1 of 1

Printed By: marwa mohamed yousse



Ministry of Health and Medical Services
Kingdom of Saudi Arabia

Ministry of Health and Medical Services
Kingdom of Saudi Arabia



MDT form

A-Patient ID:

1- Name : **مروة فتوح العبد مكرم**

Sex:

2- Medical numbers:
Female

83102059
Old Med:

509-2021

3-ID numbers:

Patient Age: 50 Y

Family History

Present

Phone

01125911101

BMI

21

REASON OF MDT

Marital status

Married

NHS Score

100

Group

1

Date of Mdt

11/09/2021

Co morbidity

None

Menopausal status

Postmenopausal

Initial Stage

1

Clinical data

Breast Imaging :

Mets Workup :

MBC to lung , med LN

DX
IDC

ER
3/B

PR
6/8

Her 2 Neu
3

SISH

Ki67

T

N

M

1 lung ,
med LN

Sonomammography

PATIENT HISTORY

received 15 wks taxol / carbo then kept on maintenance 27 cycles herceptin last 11/2022

pet ct 4/2021: free

ct : 9/2023: free rt axilla LN

pt is in CR &? on FU till 2/2024

recurrent rt breast mass BIRADS 5

patholgy : recurrence

pet ct: newly developed rt breast small metabolic active nodule

start taxol + herceptin received 10 wks + 2 cycles

ct : free

last mdt: FOR Mammasonography then reMDT (if residual, go to surgery)

CR NO MASS

PET CT CR

MDT OUTCOME & PLAN

FOR ZOLADEX FEMARA(AFTER FSII, E2), CONTINUE IV TRASTUZUMAB

SIGNATURE

MEDICAL ONCOLOGY

Signature

RADIOLOGY

Signature

SURGERY

Signature

PATHOLOGY

Signature